



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : اسماء عطا على محمد
المحافظة : اسيوط الحى / المدينة : ابنوب
رقم البطاقة : 29011172502181 رقم التليفون : 01158809445
العمر (تاريخ الميلاد) : 17-11-1990 المهنة : ربه منزل
العنوان : بني محمد
تاريخ تسجيل الإستمارة : 06-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : -----
الاسم : د/ندى حسن الوظيفة : -----
الاسم : د/نرمين رمضان الوظيفة : -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: أسماء عطا علي محمد
National ID: 29011172502181



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

اسماء
عطا على محمد

٤٨ المشروع الأمريكي - ج ١ - ب ٢٧
حلوان - القاهرة

١٩٩٠/١/٢٧ ٢٩٠ ١١ ١٧ ٢٥ ٠ ٢١ ٨١

KA6127158

٢٠٢٣/٠٦ ٢٩٠ ١١ ١٧ ٢٥ ٠ ٢١ ٨١

بلون عمل

أنثى مسلمة متزوجة
جمعته حسنى بدوى محمد

البطاقة مصرية حتى ٢٠٣٠/١٠/٠٧

١

MDT FORM

Name:	Asmaa Ata Ali Mohamed	MRN:	3067
Age:	35 Y	ID:	503/2025
Menopausal Sta	premenopausal	Phone:	01107382201
NRS SCORE:		Marital Status:	married
BMI:		Family History:	+ve
CO Morbidity:	free	Reason Of MDT:	

Clinical Data:

BX: IDC
ER: 0/8 **PR:** 0/8 **HER2:** -ve **KI67:** 60%

Sonomammography:

Mets Workup:

History:

34 YEARS OLD FEMALE PT WITH +VE FAMILY HISTORY BREAST CANCER

RT BREAST CANCER IDC TNBC KI67 60%

CT3N1M0

DDAC *4

8 WEEKS TAXOL /CARBO

SM+SLN AT 12/11/2025

PARTIAL PATHOLOGICAL RESPONSE

YPT1AYPN0

LN 0/15

RECVD 3 CYCLES XELODA LAST ONE AT 17/2/2026

PORT 3 FX FROM 15 FX AT KASR EKINY

BRCA 1 PATHOGENIC MUTATION

FOR MDT FOR OLAPARIB AND PROPHYLACTIC BILATERAL Sialpingo-oophercetomy

Recommendations:

MDT Outcome & Plan

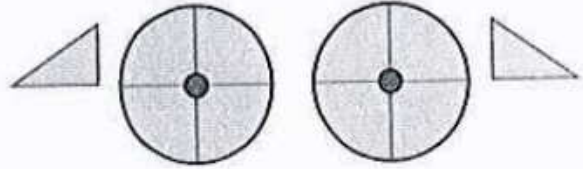
for PORT

ADJ Lynparza

counselling the patient regards PROPHYLACTIC BILATERAL Sialpingo-oophercetomy

R

L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. حنان عزت					

Name: <u>اسماء عطا علي محمد</u>		
MR. : <u>3067</u>		
Detailed Medical Report		
		السيد
تحية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد أنها		
The patient is a 34-year-old female with a positive family history of breast cancer. She was diagnosed with invasive ductal carcinoma, triple-negative breast cancer (TNBC), Ki-67 60%, clinical stage CT3N1M0. She underwent neoadjuvant chemotherapy with 8 weeks of Taxol/Carboplatin (C4) and sentinel lymph node biopsy at 12/11/2025, achieving a partial pathological response (ypT1aN0, 0/15 nodes). She subsequently received 3 cycles of capecitabine (Xeloda), the last cycle administered on 17/02/2026. A port was placed on day 3 of cycle 15 at Kasr El Einy. The patient has a pathogenic BRCA1 mutation and is being considered for olaparib and prophylactic bilateral salpingo-oophorectomy. She is currently on adjuvant Xeloda 3 × 3 × 14 for cycle 3. The patient received 5 cycles of Xeloda following neoadjuvant therapy; the treatment was stopped by the Ministry of Health. Approval for lapatinib (Lynparza) is pending; the patient may continue Xeloda until Lynparza is approved. She was counseled regarding risk-reducing surgeries, including contralateral mastectomy after completion of adjuvant therapy and salpingo-oophorectomy after age 40. Lynparza was approved by the Ministry of Health on 21/06/2026 and will be initiated once the patient presents to the clinic. The patient is currently on olaparib 300 mg twice daily (c2). She reports fatigue as the only complaint, and laboratory values are within acceptable limits. The patient has completed 2 cycles of olaparib.		
تحريراً في ,		
	التاريخ :	21-09-2026
	الوقت :	
	شيماء فاروق يحيى لاشين	الطبيب المعالج :
يعتمد ,		



PATIENT NAME اسماء عطا على محمد			Registered 04-01-2026 10:29:37	Collected 04-01-2026 13:03:33
Visit Number 72326500103			Authenticated 31-01-2026 13:04:57	Printed 02-02-2026 12:13:53
Age 35 Year	Gender Female	Referred By Prof : -	Client ID 55452	

MOLECULAR BIOLOGY REPORT

BRCA 1.2 WHOLE GENE SEQUANCING / BLOOD (Astrazneca)

Result: A pathogenic variant was detected in *BRCA1* gene by sequencing.

NM_007294.4 (*BRCA1*):c.4065_4068del (p.Asn1355LysfsTer10)

Variant interpretation:

- . A heterozygous frameshift deletion variant is detected in *BRCA1* gene that is predicted to encode a truncated nonfunctional protein.
- . The variant is a null variant in a gene where loss of function is a known mechanism of disease.
- . This premature translational stop signal has been observed in individual(s) with breast and ovarian cancer (PMID: 8571953, 11802209, 14757871, 21559243). It has also been observed to segregate with disease in related individuals. This variant is also known as 4184del4. ClinVar contains an entry for this variant (Variation ID: 17674)
- . This variant has extremely low frequency in gnomAD.
- . The variant is classified as pathogenic variant by ACMG guidelines (PS4, PVS1, PM2).

Background

- Hereditary Breast Cancer accounts for about 5-10% of female breast cancer and 4-40% of male breast cancer. The most common cause is mutations in the *BRCA1* & *BRCA2* genes which are tumor suppressor genes located on chromosomes 17 and 13 respectively.
- Pathogenic variants of these genes are inherited in an autosomal dominant manner and are associated with hereditary breast and ovarian cancer (HBOC) syndrome.
- HBOC syndrome increases the risk for breast (female and male), ovarian, fallopian tube, peritoneal, pancreatic, and prostate cancers. Additionally, *BRCA2* carriers may be at increased risk for melanoma.

Method

- Genomic DNA was extracted from the patient's specimen. Libraries were prepared according to manufacturer's protocol then next-generation sequencing was performed on the Illumina MiSeq platform using CE-IVD BRCA assay

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University

1-2



PATIENT NAME اسماء عطا على محمد			Registered 04-01-2026 10:29:37	Collected 04-01-2026 13:03:33
			Authenticated 31-01-2026 13:04:57	Printed 02-02-2026 12:13:53
Visit Number 72326500103	Age 35 Year	Gender Female	Referred By Prof : -	Client ID 55452

- The assay analyzes sequencing of all exons and immediately adjacent intronic regions of BRCA1 & BRCA2 genes.
- Raw sequence data analysis, including base calling, demultiplexing, alignment to the hg19 human reference genome (Genome Reference Consortium GRCh37), and variant calling is done using standalone software. Variants are interpreted using publicly available databases such as Exac, GnomAD, dbSNP, ClinVar, Brca exchange. Variants are classified according to the American College of Medical Genetics and Genomics (ACMG) guidelines.
- Benign and likely benign variants are not included in this report.
- Variants of unknown significance classification may change over time.

Limitations

- Large rearrangements may not be detected, variants in regions with high GC content or pseudogenes may give false negative or false positive results.
- The test result is for clinical reference and clinicians should interpret it in the context of the patient's condition, clinical findings, family history, other lab investigations, and international guidelines for treatment.

Recommendations

- Genetic counseling may be of benefit.
- At-risk family members should be offered testing for the identified pathogenic variant.

References

- Yoshida, Reiko. "Hereditary breast and ovarian cancer (HBOC): Review of its molecular characteristics, screening, treatment, and prognosis." Breast Cancer 28.6 (2021): 1167-1180.
- Richards, Sue, et al. "Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology." Genetics in Medicine 17.5 (2015): 405-423.

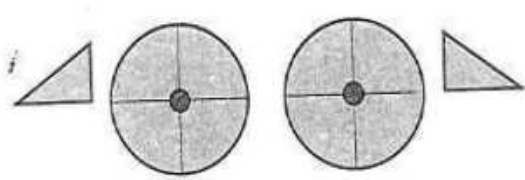
Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University 2-2

MDT FORM 21 / 12 / 2025 G

Post

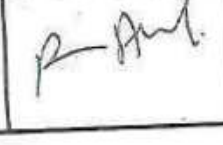
Name	احمد علي ص	MRN	503/2025
Age	35	ID	99111750-111
Menopausal status		Phone	1101119440
NRS SCORE		Marital status	
BMI		Family history	+ve
CO morbidity	free	REASON OF MDT	Post op

Clinical Data			
BX: RT UOQ 1 October 3.7.25 R (CT3M1Mo) L			
ER:	PR:	HER 2:	Ki67:
0	0	0	60%
Sonomammography: IDC G III			
Mets Workup: free			
			
DDAC x 4, 8 weeks tadol, Gerbo TNM Post NACT Post SM+SLN - free margins - L.N 0/15 ⇒ (PPR), RCB class I (XPT1a, Mo)			

MDT OUTCOME & PLAN

- An BRCA testing - if +ve Re-MDT
 - An adjuvant x 6ms + porter

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

Patient Name : الأستاذ / اسماء عطا علي محمد
Patient No. : 29011172502181
Path No. : 3481/25
Sex : Female
Age : 34 Y
Referring Doc :

Request No. : 13003196
Request Date : 12/11/2025
Verification Date : 06/12/2025
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Post-NACT.
Nature of specimen: Right simple mastectomy & Sentinel lymph nodes.

Gross Examination:

Two specimens were received as follow:

- * Right simple mastectomy specimen measured 20x14x4 cm, with covering skin ellipse measured 18x10 cm. Nipple & areola looked unremarkable grossly.
- * Serialing showed a tumor bed site measured 4x4x3 cm, located 0.2 cm away from the deep margin.
- * Sentinel lymph nodes: Multiple fibrofatty tissue pieces collectively measured 3x3 cm, entangling five lymph nodes, the largest is 1.5x1x0.5 cm.
- 10 Blocks:
- 1:7- T.B.S. (slide 1 for IPT), 8- Deep margin., 9- N&A., 10- SLNs.
- Gross was done by; Dr. Mennan Hossam.

** Frozen (intra-operative) results:
- Free five sentinel lymph nodes (0/5).

Microscopic examination:

- * **Histological type:** Minimal residual viable invasive duct carcinoma (Actual tumour size 0.5x0.5 cm) (Cellularity 2%).
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Nipple and areola: Free.
- * Deep margin: Free.
- * Sentinel lymph nodes: Five lymph nodes free of metastatic deposits (0/5).

Verified by :
Dr.Duaa Mohamed

Printing By : .marwa mohamed youss

Printing Date : 07/12/2025 09:46:28

Page 1 of 2



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / أسماء عطا علي محمد
Patient No. : 29011172502181
Path No. : 3481/25
Sex : Female
Age : 34 Y
Referring Doc :

Request No. : 13003196
Request Date : 12/11/2025
Verification Date : 06/12/2025
Financial Category : جوماتف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

* Histologic response to therapy: Pathologic partial response to therapy.

Diagnosis:

Right breast cancer (Post-NACT), Right simple mastectomy & Sentinel lymph nodes biopsy:
- Minimal residual viable invasive duct carcinoma, Grade 2 (Cellularity 2%) (Actual tumor size 0.5x0.5 cm).

- * Ductal carcinoma in situ: Absent.
- * Nipple and areola: Free.
- * Deep margin: Free.
- * Sentinel lymph nodes: Five lymph nodes free of metastatic deposits (SLNs 0/5).
- Stage: ypT1a, ypN0 (sln).
- RCB: 1.348, RCB class-I.

Signature

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

Printing Date : 07/12/2025 09:46:28

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / أسماء عطا علي محمد
Patient No. : 29011172502181
Path No. : 5201/24
Sex : Female
Age : 34 Y
Referring Doc :

Request No. : 13008743
Request Date : 26/12/2024
Verification Date : 09/02/2025
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- ER: Negative (score 0 /8).
- PR: Negative (score 0 /8).
- Her2: Negative (score 0).
- Ki67: Positive in 60% of tumor cells.
- GATA3: Very few positive cells.
- SOX10: Positive.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Amal Hareedy.
Dr. Rana Fathy.

Verified by :
DR.Menna Nabil

Printing Date : 09/02/2025 12:37:57

Page 2 of 3

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Asmaa Atta Ali Mohamed	Date : 11/10/2025
Exam : MAMMOGRAPHY BOTH	Age : 35 Year(s)
Medical ID: 503/2025	Pat. ID: 97830

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up after neo-adjuvant chemotherapy of pathologically proved right breast carcinoma.

- **Breast density:** Moderate dense breast tissue . *Breast density matches ACR C.*
- **Focal asymmetry in the right upper outer quadrant**
- suspicious mass density, architectural distortion or pathological micro-calcification in the left breast .
- Normal skin and nipple.

BREAST HIGH RESOLUTION ULTRASONOGRAPHY REVEALED:

Right breast:

- Heterogeneous glandular background parenchyma.
- **Complete disappearance of the previously described mass**
- Normal skin, subcutaneous fat and nipple areola complex.
- **Restoration of size and shape of previously described LN (Non specific LN).**

Left breast:

- Heterogeneous glandular background parenchyma.
- **No detected suspicious solid or cystic masses.**
- No detected significant left axilla
-

Conclusion:

- **Right breast:** Complete radiological response (CR) of the pathologically proved right breast UOQ malignant mass, BIRADS 6 category.
- **Left breast:** NORMAL BIRADS 2 Category.
- **DR FARAH ELSHOBAKY**

Patient Name: Asmaa Atta Ali Mohamed	Date : 03/08/2025
Exam : MRI RIGHT SHOULDER	Age : 34 Year(s)
Medical ID: 503/2025	Pat. ID: 97830

MRI OF THE RIGHT SHOULDER

Findings:

- Marrow infiltrative process seen involving the cortical part of greater tuberosity and patchy area at humeral head , displaying low T1, high PD SPIR associated with bone marrow edema , no associated soft tissue component.
- Mild infero lateral slopping of acromion
- Subtle intra substance area of intermediate T2 & high STIR signal intensity is seen involving impingement zone of supraspinatus tendon
- Subtle effusion with sub acromial sub deltoid bursa
- No areas of abnormal marrow infiltration of all shoulder bones.
- Unremarkable glenoid labrum with no sign of abnormal signal intensity.
- Normal thickness and signal intensity of the rest of rotator cuff tendons.
- Normal MR appearance of the peri-articular musculature with preserved inter-muscular fat planes.

OPINION:

- Marrow infiltrative process of greater tuberosity and humeral head.
- Mild infero lateral slopping of acromion
- Subtle Supra spinatus tendinopathy .
- Subtle effusion with sub acromial sub deltoid bursa

Dr.Reham Khedr(MD)



Patient Name: Asmaa Atta Ali Mohamed	Date : 20/02/2025
Exam : C.T ABDOMEN & PELVIS	Age : 34 Year(s)
Medical ID: 503/2025	Pat. ID: 97830

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- Right breast UQ well defined heterogenous density mass lesion measuring 5.4 X 3.6 cm.
- Right axillary level I and II enlarged lymph nodes the largest measures 2.1 x 1.3 cm.
- No sizable suspicious pulmonary nodules, masses or cavitary lesions could be seen.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen irregularities on non-contrast basis.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with right renal gravels and with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Underfilling of the urinary bladder, however no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.



Patient Name: Asmaa Atta Ali Mohamed	Date : 20/02/2025
Exam : C.T ABDOMEN & PELVIS	Age : 34 Year(s)
Medical ID: 503/2025	Pat. ID: 97830

- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Right breast mass lesion with enlarged ipsilateral axillary lymph nodes as described, for SM correlation.***
- ***No definite pulmonary, hepatic or osseous metastatic deposits.***

Dr. Maie Khattab, MSc.

Dr. Wafid Mohamed, M.D.



pp

وزارة الصحة والسكان
إدارة المراكز الطبية التخصصية
مستشفى اورام دار السلام

Echo report

Name:
Date: 9/4 2025

Age:

0.2
C2

EF	66%	54-74%	PWT	0.6	0.6-1.1cm
LVEDD	4.3	3.5-5.7cm	AO	2.1	2.0-3.7cm
LVESD		2.2-4.0cm	LA	3.0	1.9-4.0cm
FS		25-45%	RV		0.7-2.3cm (supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions: normalLV wall thickness: normalLV diastolic function: normalSWMA: normalLA size: normalAortic root dimension: normalMitral Valve: normalAortic valve: normalTricuspid valve: Trivial TRPulmonary valve: normalRight side size and systolic function: normalIntracardiac masses and thrombi: normalPericardium: normalIVC diameter and collapsibility: normal

Conclusion:

D/

د. هادي حجازي

ATAA, ASMAA Patient ID: 10110225009374 DOB: 17-Nov-1990 Date: 27-May-2025

Patient Lastname: Ataa

Patient Firstname: Asmaa

HIS Patient ID: 10110225009374

RIS Patient ID: 370181

Date of Birth: 17/11/1990

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (27/5/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP
Revealed:

" Active focal tracer uptake is seen at right humeral head ?? Severe arthritic changes ?? otherwise for CT cuts correlation.

" Degenerative changes over the dorso-lumbar vertebrae.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized.

Conclusion: -

" Equivocal right humeral head osseous lesion for CT cuts correlation to prove its nature.

Much Obligated

Merua Nader

Randa Ali, MSc

===== REPORT TEXT ENDE =====